

Patient Demographics

Name	Patient ID	SSN	Gender Identity	Birth Date	
Harden, Addilynn Rue	23623412	xxx-xx-8488	Female	03/05/26 (4 mos)	
Address	Phone	Email			
239 MIRIAM WAY MOUND HOUSE NV 89706-8248	775-224-7701 (M)	—			
Reg Status	PCP	Date Last Verified	Next Review Date		
Verified Registration	Jenny Smokey, MD775-885-2229	03/05/26	09/01/26		

Harden, Addilynn Rue #23623412 (Acct:8209382095)
(4 month old F) PCP: SMOKEY, J (775-885-2229)

01

ED Arrival Information

AD Arrival information			
Expected	Arrival	Acuity	
-	7/18/2026 18:26	Level 4	
Means of arrival	Escorted by	Service	Admission type
Private Transportation	Parent	Emergency	Emergency
Arrival complaint			
Constipation			

Chief Complaint

Complaint	Comment
Constipation	

ED Treatment Team

Provider	Role	From	To	Phone	Pager
Jennifer L. Peters, PA-C	Attending Provider	07/18/26 1856	--	775-445-8000	--
Leslie Popovich, RN	Registered Nurse	07/18/26 1911	--	--	--

ED Provider Notes**ED Provider Notes by Jennifer L. Peters, PA-C**

07/18/26 1946

Author: Jennifer L. Peters, PA-C Service: Emergency Author Type: Physician Assistant
 Filed: 07/18/26 1955 Date of Service: 07/18/26 1946 Status: Signed
 Editor: Jennifer L. Peters, PA-C (Physician Assistant)

EMERGENCY MEDICINE NOTE/ HPI:

Addilynn Rue Harden is a 4 month old female born full term, vaginal delivery who presents to the ED with concern for constipation for the last 24 hours. She apparently has been straining to pass harder than usual stools. No fevers, cough or rhinorrhea. No rashes. Appetite within normal limits. Patient is breast and formula fed without recent formula changes, the most recent formula change was about a month ago. Mom can not list any specific dietary changes for her. Vaccinations up-to-date. Already seen at urgent care earlier today, recommended to try glycerin suppositories but wanted a 2nd opinion about this. Scheduled for follow-up with primary care in two days^[JP.1]

Chief Complaint

Patient presents with

ED Provider Notes (continued)**ED Provider Notes by Jennifer L. Peters, PA-C (continued)**

07/18/26 1946

- Constipation^[JP.2]

Means of Arrival:^[JP.1] Private Transportation^[JP.2]

Nursing and triage documentation reviewed.

REVIEW OF SYSTEMS:**Review of Systems**

Constitutional: Negative for crying and fever.

HENT: Negative for congestion and rhinorrhea.

Respiratory: Negative for cough.

Gastrointestinal: Positive for **constipation**. Negative for vomiting.

Skin: Negative for rash.

All other systems were negative, mentioned in HPI, or non-contributory to this visit^[JP.1]

History reviewed. No pertinent past medical history.^[JP.2]

PCP:^[JP.1] Jenny Smokey

No past surgical history on file.

Family History

Problem	Relation	Name	Age of Onset
• Depression	Mother	Nearhoff, Hanna Ariel	

Copied from mother's medical history at birth^[JP.2]

Social History

^[JP.1]

No current outpatient medications

Outpatient medications were reviewed.^[JP.1]

No Known Allergies^[JP.2]

PHYSICAL EXAM:

Vital signs reviewed.^[JP.1]

Vitals

	07/18/26 1843
Pulse:	141
Temp:	36.9 °C (98.4 °F)
Resp:	30

ED Provider Notes (continued)**ED Provider Notes by Jennifer L. Peters, PA-C (continued)**

07/18/26 1946

Weight: 5.5 kg (12 lb 2 oz)

SpO2: 94%^[JP.2]

48.9 cm (1' 7.25")

^[JP.1] 12 lbs 2 oz^[JP.2],^[JP.1] There is no height or weight on file to calculate BMI.^[JP.2]Physical ExamConstitutional:

General: She is active.

Appearance: Normal appearance. She is well-developed.

Comments: Nontoxic very well-appearing infant lying on ED gurney. Parents at bedside. She is smiling cooing and interactive

HENT:

Head: Normocephalic and atraumatic. Anterior fontanelle is flat.

Right Ear: Tympanic membrane, ear canal and external ear normal.

Left Ear: Tympanic membrane, ear canal and external ear normal.

Nose: Nose normal. No congestion or rhinorrhea.

Mouth/Throat:

Mouth: Mucous membranes are moist.

Pharynx: Oropharynx is clear. No oropharyngeal exudate or posterior oropharyngeal erythema.

Eyes:

Conjunctiva/sclera: Conjunctivae normal.

Pupils: Pupils are equal, round, and reactive to light.

Cardiovascular:

Rate and Rhythm: Normal rate and regular rhythm.

Heart sounds: Normal heart sounds.

Pulmonary:

Effort: Pulmonary effort is normal. No retractions.

Breath sounds: Normal breath sounds. No stridor.

Abdominal:

General: Bowel sounds are normal. There is no distension.

Tenderness: There is no abdominal tenderness.

Comments: Abdomen soft nontender without distention. Normoactive bowel sounds.

Genitourinary:

Comments: No anal fissures noted. Mild erythema around the anus

Musculoskeletal:

General: Normal range of motion.

Cervical back: Normal range of motion and neck supple.

Skin:

General: Skin is warm and dry.

Capillary Refill: Capillary refill takes less than 2 seconds.

Turgor: Normal.

Neurological:

Mental Status: She is alert.

Primitive Reflexes: Suck normal.

ED Provider Notes (continued)**ED Provider Notes by Jennifer L. Peters, PA-C (continued)**

07/18/26 1946

RESULTS:^[JP.1]

No results found for this or any previous visit (from the past 48 hours).

XR ABDOMEN 1 VIEW

Result Date: 7/18/2026

Mild gaseous distention of the colon and bowel.^[JP.2]

ED COURSE:^[JP.1]

Clinical Impressions as of 07/18/26 1955

Constipation, unspecified constipation type^[JP.2]

PROCEDURES:

Procedures

EKG: N/A

MEDICAL DECISION MAKING:

Addilynn Rue Harden is a 4 month old female born full term, vaginal delivery who presents to the ED with concern for constipation for the last 24 hours. She apparently has been straining to pass harder than usual stools. No fevers, cough or rhinorrhea. No rashes. Appetite within normal limits. Patient is breast and formula fed without recent formula changes, the most recent formula change was about a month ago. Mom can not list any specific dietary changes for her. Vaccinations up-to-date. Already seen at urgent care earlier today, recommended to try glycerin suppositories but wanted a 2nd opinion about this. Scheduled for follow-up with primary care in two days. Vitals are stable currently. On exam patient is awake alert quite well-appearing smiling interactive and cooing on ED gurney. Lungs are clear throughout. No stridor or retractions noted. ENT exam benign, moist mucous membranes and TMs within normal limits bilaterally. Fontanelle is flat. Abdomen soft nontender without distention. Normoactive bowel sounds noted. One-view x-ray of the abdomen was ordered earlier today from urgent care at renown this showed mild gaseous distention of the colon and bowel without free air. I do not feel a need to repeat imaging at this time. Patient is really quite well-appearing, she is smiling cooing interactive. Recommend over-the-counter pediatric glycerin suppositories and also discussed the option of adding 2 oz of 100% fruit juice apple/paresthesias/prune to baby's formula or breast milk in a 24 hour period. I also discussed with parents placing a lubricated rectal thermometer or small portion of a pinky finger into the rectum to encourage/stimulate bowel movement. No indication for any further workup today, patient is quite well-appearing. Fortunately she already has follow-up scheduled with primary care and just two days. ED return precautions discussed and understood.

Given the careful history/physical and the entirety of the evaluation, I feel that the patient at this time is stable for further outpatient follow-up regarding the issue we discussed. The plan and thought processes were discussed at length with the patient. Patient agrees to obtain follow-up as discussed at length. If the patient needs a referral to outside specialist or primary care, referral was provided. I instructed the patient on ED

ED Provider Notes (continued)**ED Provider Notes by Jennifer L. Peters, PA-C (continued)**

07/18/26 1946

return precautions if there are any new, worsening or otherwise concerning symptoms. Patient expressed understanding and agreement with this plan verbally. Opportunity was given for questions prior to discharge and all stated questions were answered to the patient's satisfaction. Upon discharge, vital signs were reassessed by me and reconciled. (This note was partially dictated utilizing M modal technology. Effort was made to proof read the document prior to signing, however grammatical and/or contextual errors are still possible and may be present after signing)

Patient was counseled in person regarding the patient's stable condition, test results, diagnoses and need for additional testing and follow-up as discussed. Patient agrees with plan of care. Concerns were addressed.

DIFFERENTIAL DIAGNOSIS: SBO, constipation, anal fissure, diaper dermatitis, Hirschsprung disease

MIPS:

Blood pressure was reviewed. Patient was counseled on the importance of follow-up with her primary care provider regarding further blood pressure monitoring management is appropriate.

Tobacco use history reviewed and appropriate counseling/materials available were provided were indicated.

Sexually transmitted infection testing/screening was discussed/offered to/with the patient, N/A

DIAGNOSTICS:

Laboratory (if available/applicable) and imaging studies (if available/applicable) were independently reviewed and interpreted by me. These include N/A.

CONSULTATIONS: QHP consultation with N/A to discuss workup/disposition/recommendations. They recommend N/A

EXTERNAL RECORD REVIEW: Urgent care record reviewed as above

MEDICAL MANAGEMENT (IF APPLICABLE): The patient required prescription management including glycerin suppository

PROCEDURE/ SURGERY PLANNED/ DISCUSSED: N/A

HISTORY: History is obtained primarily from the patient's mother and father in order to gain clinically pertinent information

SOCIAL HISTORY: Based on my evaluation, diagnosis/treatment of the patient's condition is significantly limited by certain social determinants of health including none pertinent

DISPOSITION: Discharged home

ED Provider Notes (continued)**ED Provider Notes by Jennifer L. Peters, PA-C (continued)**

07/18/26 1946

NEW MEDS / PRESCRIPTIONS:^[JP.1]**Patient's Medications**No medications on file^[JP.2]**DIAGNOSES:**^[JP.1]**Final diagnoses:**Constipation, unspecified constipation type^[JP.2]**DISPOSITION:**^[JP.1]

Discharge

Jen Peters, PA-C^[JP.2]

Emergency Medicine

Attending physician Dr. Doering was available to discuss case/workup/findings of above patient.

Jennifer L. Peters, PA-C

07/18/26 1955

^[JP.2]**Attribution Key**

JP.1 - Jennifer L. Peters, PA-C on 07/18/26 1946

JP.2 - Jennifer L. Peters, PA-C on 07/18/26 1955

ED Course

There is no data filed.

Dictations

None

ED Diagnosis

Diagnosis	Description	Comment	Associated Orders
Final diagnosis			
Constipation, unspecified constipation type	Constipation, unspecified constipation type	--	--

ED Disposition

ED Disposition	Condition	Comment
Discharge	--	--

Results

None

Imaging Results

None

ECG Results

None

Home Medications

No medications reported.

Medication Administration from 07/18/2026 1826 to 07/18/2026 1957

None

ED Prescriptions

None

ED Medication Orders (From admission, onward)

None

Code, Iso, Restraint (From admission, onward)

None

ED Imaging Orders (From admission, onward)

None

ED Micro, Lab, POCT (From admission, onward)

None

ED All Orders (From admission, onward)

None

Discharge Orders (From admission, onward)

None

Allergies (Reviewed on: 07/18/26)

No Known Allergies

Tetanus Up To Date

None

Medical History

No past medical history on file.

Surgical History

No past surgical history on file.

Social History

Other Factors

Not Asked

Social Documentation

No social documentation on file.

Family Medical History

This family history was copied from the mother's chart. Sibling and father relationships might require correction.

Problem	Relation	Age of Onset	Comments
Depression	Mother		Copied from mother's medical history at birth

ED LDA Documentation (7/17/2026 00:00 to 7/18/2026 19:57)**ED Events**

Date/Time	Event	User	Comments
07/18/26 1825	Emergency encounter created	SERRANO, NICOLE	--
07/18/26 1826	Patient arrived in ED	SERRANO, NICOLE	--
07/18/26 1837	Registration Completed	SERRANO, NICOLE	--
07/18/26 1841	Triage Started	SALCI, NATALIE	--
07/18/26 1843	Triage Completed	SALCI, NATALIE	--
07/18/26 1856	Assign Attending	PETERS, JENNIFER L	Jennifer L. Peters, PA-C assigned as Attending
07/18/26 1856	Assign Physician	PETERS, JENNIFER L	--
07/18/26 1904	Patient roomed in ED	CARR, HANNAH	To room VC
07/18/26 1911	Assign Nurse	POPOVICH, LESLIE	Leslie Popovich, RN assigned as Registered Nurse
07/18/26 1925	Patient transferred	POPOVICH, LESLIE	--
07/18/26 1957	ED Tracking End	POPOVICH, LESLIE	--
07/18/26 1957	Patient discharged	POPOVICH, LESLIE	--

Follow-up Information

Follow up With	Specialties	Details	Why	Contact Info
Jenny Smokey, MD	PEDIATRICS	In 2 days		1475 Medical Pkwy Carson City NV 89703-4635 775-885-2229

Discharge Instructions

Please follow-up with a primary care provider in the next 2-3 days. Return to the ED immediately or call 911 with any new or worsening symptoms. Thank you for choosing our emergency department today, it was a pleasure caring for you

You may use over the counter PediaLax glycerin suppositories to help with occasional constipation in addition you may also give 1-2 oz of 100% fruit juice with formula or breast milk in a 24 hour period. This would include 100% pear/prune or apple juice. A lubricated pinky finger or rectal thermometer placed gently in the rectum can also help stimulate stool passage. Follow-up as scheduled with your primary care provider in two days.

Discharge References/Attachments**Constipation, Infant, KidsHealth (English)****AVS Reports**

Date/Time	Report	Action	User
7/18/2026 7:46 PM	ED Proxy After Visit Summary	Printed	Jennifer L. Peters, PA-C

Communication Routing History

None

ED Charges

Description	Code	Date	Service Prov	Modifiers	Qty	Status
HB EMERGENCY DEPT VISIT 2	6000004	07/18/2026	Popovich, Leslie		1	Filed